

January 9, 2025

Jim O'Neill
Deputy Secretary
U.S. Department of Health and Human Services

Steven Posnack
Principal Deputy Assistant Secretary for Technology Policy
Office of the National Coordinator for Health Information Technology
U.S. Department of Health and Human Services

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care (RIN 0955-AA13)

Dear Deputy Secretary O'Neill and Principal Deputy Assistant Secretary Posnack:

Lumeris is pleased to provide our insights in response to the Department of Health and Human Services (HHS) Request for Information (RFI) regarding the clinical use of Artificial Intelligence (AI). As a pioneer in value-based care (VBC) and the developer of Tom™—our agentic AI platform—we believe that access to high quality primary care is the bedrock of a high functioning, affordable healthcare system. Given the declining provider workforce, the future of U.S. primary care will mandate the smart use of modern technologies to maximize the reach of a highly constrained human capacity.

Our Primary Care as a Service (PCaaS) model orchestrated through Tom treats AI not as a static software tool, but as a dynamic Digital Member of the Care Team capable of performing "digital labor" to scale clinical capacity, reduce provider burnout, and provide continuous, "always-on" care. By enabling agentic AI to handle "in-between" care—including proactive monitoring, patient education, and autonomous scheduling—we can ensure that primary care is accessible to every American, regardless of geographic or physician headcount constraints.

What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

The primary barriers to innovation include data fragmentation, software costs without sufficient return on investment (ROI) to warrant investment, misaligned financial incentives, and significant change management. Current fee-for-service (FFS) models fail to recognize and appropriately compensate the value of "digital labor."

Furthermore, Agentic AI requires a comprehensive, real-time view of the patient to be effective. Healthcare data is messy. Accessing, aggregating, and normalizing disparate

data sets are required for the highest use of clinical grade AI. The lack of standardized, real-time access to Pharmacy Benefit Manager (PBM) data and Admission, Discharge, Transfer (ADT) feeds, are prime examples of key strategic hurdles that prevent AI from acting on critical events (e.g., a medication non-fill or an ER visit) at the moment of highest impact. AI is a relatively expensive technology and the proliferation of point solutions that create limited value individually presents challenges to already-strained provider budgets. Platform solutions promise greater value but come with a significant initial investment in data aggregation and software implementation. Finally, clinical workflow-disruptive technologies require significant change management, typically with an associated reduction in short-term productivity.

What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

It is paramount that HHS modernize the Physician Fee Schedule (42 CFR § 414) to move beyond a human-only, time-based reimbursement paradigm. In support of fostering a modern and forward-thinking health care ecosystem Lumeris recommends HHS take the following actions:

- **Evolution of Coordination Codes:** Current codes for Remote Physiological Monitoring (RPM - CPT 99457, 99458), Chronic Care Management (CCM - CPT 99490), and Transitional Care Management (TCM - CPT 99495, 99496) require thresholds of human clinical staff time. HHS should allow AI-driven "digital labor" to fulfill a portion of these requirements. If an AI monitors data and handles patient education, the clinical objective is met more efficiently than via manual outreach.
- **Modernizing payment for "Digital Labor":** HHS should prioritize the creation of reimbursement codes for Asynchronous AI-Led Patient Engagement. Currently, if Tom successfully triages a patient or closes a diabetic care gap through autonomous outreach, there is often no mechanism for the provider to be reimbursed for that "digital work," despite the clear clinical value.

Furthermore, it is essential that HHS take steps to enable the most efficient interfaces between AI software solutions and electronic health records to foster clinical adoption:

- **Information Blocking (45 CFR Part 171):** Define the withholding of transactional APIs (scheduling, referrals, tasking) by EHR vendors as Information Blocking. For AI to fulfill care coordination, it must have the authority to act (e.g., schedule a follow-up) without proprietary gatekeeping or "innovation taxes" imposed by legacy vendors.

We encourage HHS to collaborate with the Food & Drug Administration (FDA) to modernize and clarify regulatory pathways that support safe adoption of clinically trustworthy AI. Together, HHS and FDA can scale the Predetermined Change Control Plan (PCCP) framework to better accommodate adaptive AI systems. Continued clarification of FDA's Software as a Medical Device (SaMD) guidance would further streamline compliance for developers and providers. In addition, the agencies could jointly establish FDA-supervised "clinical AI sandboxes" with academic medical centers, enabling real-world testing, structured monitoring, and iterative improvement before full clearance. Finally, HHS and FDA can outline and expand enforcement discretion policies for lower-risk AI-supported clinical activities, ensuring regulation is risk-based and does not inhibit innovation where there is low potential for harm.

For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

We strongly urge HHS to lead the creation of a Federal Safe Harbor Framework for "Augmented Intelligence." This should clarify that when AI provides contextual clinical decision support (CDS) to a "human-in-the-loop," the legal standard of care remains centered on the clinician's judgment. This reduces the "liability chilling effect" that currently slows adoption due to the defensive stance in which it places innovation.

For non-medical devices, what are the most promising AI evaluation methods (pre- and post-deployment), metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care? Should HHS further support these processes? If so, which mechanisms would be most impactful (e.g., contracts, grants, cooperative agreements, and/or prize competitions)?

As a technology driven solution, AI presents the unique opportunity to generate more real-time feedback from the end user, thereby enabling developers to address feedback in a prompt manner. Our Tom platform leverages clinician-led feedback through a "thumbs up/down" mechanism that allows the provider to validate or challenge AI generated recommendations in real-time. This communication avenue is closely monitored as an indicator of logical drift or regional bias, allowing us to make modifications, as appropriate, based on the provider's feedback.

AI tools have the distinct ability to provide multiple layers of oversight through other AI-powered solutions. Lumeris utilizes secondary, high-reasoning large language models (LLMs) to perform automated audits of Tom's patient interactions (an LLM-as-a-Judge). This "Judge" has the capability of flagging safety concerns or other aberrancies for human review, serving as a collaborative partner in the AI monitoring process. Such technologies have the incredible opportunity of boosting monitoring

productivity compared to largely manual efforts that require significant human time and resources.

With the goal of ensuring consistency across the industry, we strongly encourage HHS to fund the development of “Gold Standard” clinical judge datasets that could be leveraged by vendors to benchmark the safety of their LLMs. In the absence of a universal standard, the quality of AI tools provided to clinicians may vary broadly, which in turn will create differential experiences by patients across the board. Safety must remain at the center of all clinical interactions, regardless of the mechanisms through which they are administered.

How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

HHS should promote Outcome-Based Certification that prioritizes the performance of AI tools in real-world environments (e.g., impact on HEDIS scores) rather than solely on static software audits. Real-world evidence provides a more meaningful assessment of the performance of these tools compared to non-realistic “lab-based” environments.

Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

Tom has proven that patients of all ages are willing and able to engage with a conversational AI interface. Tom outperforms simple reminders, taking that next step in educating, handling objections, and autonomously scheduling services. As a Digital Member of the Care Team, it reduces practice phone volumes and in-basket messages while triaging patients to the most efficient care venue, creating additional touchpoints to improve quality of care and patient outcomes.

Contextual Clinical Decision Support (CDS) represents one of the greatest novel AI use-cases in the near term. Such applications allow for the upskilling of providers by connecting a 360-degree patient view to real-time, evidence-based recommendations (e.g., UpToDate®). Presenting insights natively at the point of care ensures compliance with the latest guidelines, improving clinical outcomes.

Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

Making the decision to deploy significant resources, including AI solutions, typically requires the attention and approval of an organizations Chief Financial Officer (CFO), tasked with the ever-challenging role of prioritizing investments within the context of overall organizational financials, strategic objectives, and ROI. The Chief Medical Officer (CMO) and Chief Information Officer (CIO) are other primary stakeholders,

responsible for ensuring the clinical goals of the organization are being met and that technology solutions comply with the relevant requirements. Increasingly, health systems with the necessary resources may take the additional step of naming a Chief Health AI Officer to provide dedicated focus on the implementation and ethical use of AI.

The largest administrative clinical hurdle reducing the adoption of AI in clinical contexts is the "Workflow Interruption Cost"—if AI requires a provider to leave their native EHR environment, adoption fails. It is vital that AI solutions integrate smoothly with existing EHR environments and regulatory policies ensure that the competitive landscape of healthcare technology is not dominated by a monopoly of a few EHR provider's proprietary AI tools. Additional barriers include newer AI governance structures with lengthy privacy and legal reviews as well as IT resourcing for data acquisition and integration efforts.

Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

Interoperability must be a cornerstone of an AI-powered healthcare technology ecosystem and transition from "Read-Only" to "Transactional." At present, most EHR vendors allow an AI to "read" data via Fast Healthcare Interoperability Resources® (FHIR) but gatekeep the proprietary application programming interfaces (APIs) needed to "write" actions like scheduling or ordering a referral. HHS must mandate non-discriminatory access to Action APIs to allow AI agents to perform the digital labor necessary to reduce administrative impact, unlocking the full return from these burden reduction capabilities.

What challenges within health care do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Patients and caregivers increasingly seek connectivity and "always-on" access to their providers while also having the primary concern of losing the "human touch." Lumeris addresses this by ensuring Tom always functions as a member of the team, escalating complex or emotional concerns to human nurses or staff immediately. Tom works from patient-specific data, personalizes patient interactions, and has access to schedule or prompt proactive active outreach from the human care team, striking the balance between high-powered connectivity and sensitivity in interaction.

In addition to loss of human connection, patients and caregivers can be hesitant to adopt AI and other technology in healthcare settings due to concerns related to the privacy and security of their health information, the safety and reliability of clinical judgment and decision-making, and potential for bias and equity concerns. These concerns can be addressed through promotion of digital health literacy initiatives by HHS, providers, and the broader healthcare community. Ensuring healthcare AI

complies with existing and new requirements for privacy and security establishes greater trust in technology, and transparency in clinical decision-making and published research on improved outcomes can assure the public of the benefits of AI technology in healthcare across clinical settings, geographic regions, and demographic groups.

Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

We would strongly advise HHS to prioritize Implementation Science in driving the adoption of AI in clinical care settings. Equally important, AI-enabled models of care should be designed and evaluated to ensure consistent access to high-quality care across diverse practice settings and patient populations, particularly in rural, underserved, or workforce-constrained communities.

Research is needed to understand how the integration of digital resources that expand provider capacity shifts the economics of primary care and impacts long-term health outcomes, particularly among underserved populations. Attention should be given to the role of practice size and affiliation with health systems and state-based university systems in the availability and implementation of AI tools and their impact on patient experience. It is essential that HHS understand the financial and technological limiting factors that may reduce the ability for all providers, and all patients, to experience the benefits of AI adoption in the U.S.

Conclusion

Lumeris appreciates the opportunity to provide our insights as a healthcare technology company dedicated to transforming primary care through AI-driven solutions. We are ready and enthusiastic to partner with HHS in transitioning the U.S. healthcare system into the era of Primary Care as a Service where every American has continuous access to primary care, regardless of geography or workforce shortages. By unlocking transactional data, modernizing reimbursement to account for digital labor, and adopting hybrid evaluation models, we can ensure AI is a deflationary force resulting in engaged physicians, healthy patients, stronger performance for health systems, and a healthier America.